

ENGLISH FOR WORK / SEPTEMBER 2026

Medical Laboratory Technician English



Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical laboratory technicians and clinical laboratory support staff.

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 A barcode scan and a spoken reference disagree

02 A rejection reason copied too broadly

03 An update with no invented failure cause

04 A restart estimate is not a reporting estimate

05 Two laboratories quote different intervals

06 The old printout remains in circulation

07 An interrupted notification resumed accurately

08 A send-out result arrives after shift change

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

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FULL DIALOGUE 01 / 12 SPEAKING TURNS

A barcode scan and a spoken reference disagree

Fictional laboratory desk enquiry; a scan and telephone note point to different accessions.

Roles: Technician / Receiving lead

Technician: The scan retrieves B207, but the telephone note says B270. I haven't linked the enquiry to either.

Receiving lead: Which source produced each reference? We need to separate the scan from the caller's spoken information.

Technician: The scanned document retrieves B207. A colleague wrote B270 during the call, without a read-back.

Receiving lead: That gives us a discrepancy to check, not proof the caller or the scanner was wrong.

Technician: Could you review the document association while I ask the colleague to clarify the original enquiry?

Receiving lead: Yes. I will check the registered source, and you can establish what the caller actually requested.

Technician: Should we answer the receipt question for B207 because it is the only visible scan?

Receiving lead: Not until the enquiry is identified. A visible reference isn't automatically the reference the caller intended.

Technician: I'll explain that we are clarifying two different identifiers and haven't established their relationship.

Receiving lead: Please keep both sources visible in the review, rather than overwriting one reference with the other.

Technician: Then I'll request a complete read-back from the caller through the verified contact route.

Receiving lead: I'll take the record-association check. We can answer the status question once the intended case is established.

Notice the professional language

'Which source produced each reference?' separates evidence before deciding what went wrong.

'Not proof' limits a conclusion without denying that a real discrepancy exists.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 02 / 12 SPEAKING TURNS

A rejection reason copied too broadly

Fictional laboratory communication review; a message generalized one suitability decision to an entire accession.

Roles: Receiving lead / Technician

Receiving lead: Your draft message says all testing was rejected, but the documented decision names only request M4.

Technician: I copied the reason into an accession-wide message. That makes its scope broader than the decision.

Receiving lead: Has the message gone to the clinic, or can we correct it before anyone relies on it?

Technician: It hasn't been sent. I can revise it to name M4 and leave other requests separate.

Receiving lead: Please state the documented reason for M4 without adding a cause or instructions for recollection.

Technician: I'll use insufficient quantity for M4. The other request is still awaiting its own suitability review.

Receiving lead: That distinction matters. Shared accession paperwork doesn't establish the same outcome for every requested test.

Technician: Would it help to put M4 first, so the clinic sees which request the explanation concerns?

Receiving lead: Yes. Start with the reference, then the reason, then the follow-up the lead actually requested.

Technician: I'll avoid saying collection failed. That could sound like a finding about the collector's performance.

Receiving lead: Exactly. We know the suitability decision, not who or what caused the available amount.

Technician: The revised message is now limited to M4, with the other request's review still pending.

Notice the professional language

insufficient quantity - A specimen amount that does not meet the laboratory's requirements for the requested testing.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 03 / 12 SPEAKING TURNS

An update with no invented failure cause

Fictional shift discussion about an unresolved run review; cause and scope remain unconfirmed.

Roles: Incoming technician / Outgoing technician

Incoming technician: The handoff says analyzer failure, but I only see an unresolved quality-control review. What is confirmed?

Outgoing technician: The review is unresolved. I shouldn't have named an analyzer failure without a finding from the lead.

Incoming technician: Has the lead identified any cause, or are we still at the stage of assessing the concern?

Outgoing technician: No cause has been communicated. Release is not authorized, and the review remains with Ana.

Incoming technician: Then I need the run reference, current status, and reviewer, without a guessed technical explanation.

Outgoing technician: The reference is R9. Testing is complete, release is pending, and Ana has accepted the review.

Incoming technician: Does that status apply to every run today, or only R9 as currently documented?

Outgoing technician: Only R9 is named in this handoff. I haven't established any wider effect.

Incoming technician: I'll keep that scope explicit when the clinic asks why its report isn't yet available.

Outgoing technician: Please also correct my earlier failure wording with the afternoon team; they may have repeated it.

Incoming technician: I will. We'll describe the unresolved review and refer cause questions to Ana instead of speculating.

Outgoing technician: Thank you. The next handoff can preserve the actual evidence without teaching an unverified explanation as fact.

Notice the professional language

'Only R9' keeps a local issue from becoming a laboratory-wide claim.

A named reviewer establishes ownership, not the result of the review.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 04 / 12 SPEAKING TURNS

A restart estimate is not a reporting estimate

Fictional planning discussion; instrument service may finish at noon, but report timing is unconfirmed.

Roles: Technician / Section lead

Technician: The service team estimates finishing at noon. The clinic asked whether its reports will arrive at noon too.

Section lead: Those are different estimates. Finishing service doesn't establish return to testing or the release time for reports.

Technician: What can I say about the relationship without describing technical steps that haven't been confirmed?

Section lead: Say service completion is estimated for noon, while laboratory readiness and report timing remain unconfirmed.

Technician: The clinic is planning its afternoon list. Could you own the next operational update for them?

Section lead: Yes. I can update them at twelve thirty with the status available, even if service runs later.

Technician: I'll identify twelve thirty as an update time, not a promise that testing has resumed.

Section lead: Please also identify the affected section. Other sections' availability should not be inferred from H2's downtime.

Technician: Only the hematology section using H2 is named in this enquiry; I haven't reported broader unavailability.

Section lead: Good. We need a precise scope and an honest timing statement, not a laboratory-wide closure message.

Technician: I'll pass on the service estimate, your update commitment, and the still-unconfirmed report timing separately.

Section lead: That gives the clinic useful planning information without treating one estimated event as every later stage's completion.

Notice the professional language

'Those are different estimates' separates maintenance timing from patient-report timing.

'Even if' makes an update commitment independent of whether service finishes.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 05 / 12 SPEAKING TURNS

Two laboratories quote different intervals

Fictional laboratory-service comparison; local interval starts at receipt, referral laboratory interval starts at accession acceptance.

Roles: Clinic coordinator / Technician

Clinic coordinator: Your service quotes two hours, while the referral laboratory quotes two days. Can we compare those directly?

Technician: Only after checking what each interval covers. Our quoted clock starts at receipt and ends at reporting.

Clinic coordinator: The referral laboratory's note says two days from accession acceptance. Transport happens before that, doesn't it?

Technician: Yes, transport isn't included in that stated interval. Acceptance may also occur after the parcel arrives.

Clinic coordinator: Then saying two days from our collection would leave out the earlier stages and their uncertainty.

Technician: Exactly. We need collection, transport, receipt, acceptance, and the reporting estimate kept distinct in the explanation.

Clinic coordinator: Could you confirm which laboratory actually performs this request rather than assume the shorter interval applies?

Technician: This request is referred out. The local two-hour interval is for a different service, not this one.

Clinic coordinator: I'll explain that without implying the referral laboratory is late before its defined starting point.

Technician: Please also avoid treating the quoted interval as a guarantee; no case-specific reporting date is confirmed.

Clinic coordinator: The useful update is referred testing, acceptance status pending, and no verified report date yet.

Technician: That matches the information available. I'll check the acceptance status with the referral coordinator next.

Notice the professional language

referral laboratory - An external laboratory that performs testing sent from another laboratory or service.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 06 / 12 SPEAKING TURNS

The old printout remains in circulation

Fictional report amendment follow-up; the electronic record changed but a clinic has an older paper copy.

Roles: Technician / Clinic nurse

Technician: The amended report is available electronically, but your message appears to quote yesterday's version.

Clinic nurse: You're right. I used the paper printout from the consultation folder rather than checking the current report.

Technician: The issue isn't simply whether the file opened; we need the team to recognize the version change.

Clinic nurse: I can alert the clinician and identify the newer issue time through our report-management process.

Technician: Please confirm that the amended report reaches the responsible clinician, not just the general reception inbox.

Clinic nurse: I'll take that notification task. I won't mark the clinical implications discussed before the clinician has reviewed them.

Technician: Thank you. Has anyone given the patient an explanation based on the earlier report?

Clinic nurse: I don't know yet. I'll check rather than assume the older printout was never used.

Technician: That uncertainty matters to follow-up. Please pass it to the clinician without making a clinical conclusion yourself.

Clinic nurse: I will. The current actions are to identify the amendment, reach the clinician, and clarify any earlier communication.

Technician: I'll record your acceptance of the notification and keep the patient's discussion status separate.

Clinic nurse: That is accurate. I'll confirm the notification outcome, including anything that remains unresolved after contact.

Notice the professional language

amended report - A report revised after issue, with the change recorded through the laboratory's correction process.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 07 / 12 SPEAKING TURNS

An interrupted notification resumed accurately

Fictional Lumen case D9: calcium 6.2 mg/dL, meaning milligrams per deciliter, is designated critical by this laboratory. Dr Rao's authority and case identity are verified. Report issued 09:20; call at 09:24; contact extension 418. The connection briefly obscures the value. No universal threshold, diagnosis, or treatment is implied.

Roles: Technician / Dr Rao

Technician: Dr Rao, Noor calling from Lumen laboratory with a critical result for D9. The analyte is calcium.

Dr Rao: The connection interrupted your next words. I heard calcium, but not the value or units.

Technician: I'll repeat the complete result: D9, calcium, six point two milligrams per deciliter.

Dr Rao: D9, calcium, six point two milligrams per deciliter. Please confirm that against the current report.

Technician: That matches the report issued at nine twenty. Have you received the complete result now?

Dr Rao: Yes, I have. Please record Dr Rao as the recipient; I'm on extension 418 this morning.

Technician: Extension four one eight, thank you. I'll record your acknowledgment and the confirmed read-back.

Dr Rao: Does nine twenty describe when the report was issued, or when you reached me on this line?

Technician: Nine twenty is the issue time. This call is at nine twenty-four, which I'll record separately.

Dr Rao: That's correct. My first connection dropped before I heard the value, but I have the complete result now.

Technician: I'll note the interruption and the completed notification at nine twenty-four, with your contact extension.

Dr Rao: Thank you. I have D9's calcium result as six point two milligrams per deciliter and will review the report.

Notice the professional language

critical result - A result meeting the laboratory's established criteria for prompt notification through its designated clinical pathway.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 08 / 12 SPEAKING TURNS

A send-out result arrives after shift change

Fictional handoff correction: external report is received, local review and clinic notification remain pending.

Roles: Incoming technician / Outgoing technician

Incoming technician: Your handoff says the send-out report is outstanding, but it arrived ten minutes ago.

Outgoing technician: Thank you. My handoff predates that arrival. Has the local reporting review or clinic notification occurred?

Incoming technician: Neither is confirmed yet. The document is received, but I don't want to call the whole task complete.

Outgoing technician: That's the right distinction. Please update the arrival status while leaving review and notification as separate open items.

Incoming technician: The clinic was expecting a status call at four. I accepted that task when I came on shift.

Outgoing technician: Then your update can report receipt without offering interpretation or implying the local review is finished.

Incoming technician: Should I use the external report's final label as permission to mark our notification task complete?

Outgoing technician: No. That label describes the external report; it doesn't prove our receiving clinician has been notified.

Incoming technician: I'll contact the local reviewer and retain the four o'clock call with whatever status is confirmed.

Outgoing technician: Please identify the report reference and issue time so the reviewer can distinguish it from earlier correspondence.

Incoming technician: I have those details in the handoff record. Receipt is now complete; local review and contact remain open.

Outgoing technician: I'll stop describing the external report as missing. You own the agreed update and the review-status enquiry.

Notice the professional language

'Predates' explains why a handoff may need updating without implying it was originally false.

A final external report does not itself close local review or notification tasks.

Rehearse this dialogue with an AI partner

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Accessioning and unambiguous identifiers

SHARED CASE

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Role A | Responding professional

Clarify similar laboratory identifiers without merging cases. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Clinic caller: your note says A341; check the actual request when asked.

Useful expressions

Your enquiry says ____; the receiving log lists ____.

Could you read the complete ____ back?

We have not yet established that ____ refers to ____.

Cover this until after your first attempt

Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

Second round

Say both cases came from the same clinic and ask whether that is enough to match them.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Specimen suitability and recollection language

SHARED CASE

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

Role A | Responding professional

Explain suitability wording without interpreting health status or teaching collection. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Clinic coordinator: you think insufficient quantity refers to a low measured result.

Useful expressions

___ describes the specimen, not a patient result.

The documented decision for request ___ is ___.

The care team needs to confirm ___.

Cover this until after your first attempt

For Q6, insufficient quantity describes the amount of specimen available, not a low result in the patient's blood. The laboratory lead has requested recollection through your clinic. There is no reportable result from this request in this case. Please confirm that the care team has received the recollection message; the collection arrangements belong with them.

Second round

Ask whether the collector definitely caused the problem; the cause is not established.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Quality-control status and reportable results

SHARED CASE

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

Role A | Responding professional

Explain why a completed run is not necessarily a released report. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Clinic caller: you need a status but initially equate a finished run with final results.

Useful expressions

The run is complete; ___ remains unresolved.

Completion does not establish ___.

I can confirm ___, but cannot report ___ as final.

Cover this until after your first attempt

The analytical run has finished, but the quality-control review remains unresolved and release has not been authorized. I cannot describe these results as final just because the instrument completed its work. The laboratory lead is reviewing the issue. I can report that status to your team, but I do not have a confirmed release time.

Second round

Ask whether the quality review proves the patient results are inaccurate.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Reagents, instruments, and service coordination

SHARED CASE

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

Role A | Responding professional

Coordinate equipment status without giving technical operating instructions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Laboratory lead: you have not confirmed return to service; ask what the technician needs for the clinic update.

Useful expressions

The service visit ended at ____; ____ remains unconfirmed.

Could you confirm whether ____ is available for ____?

Who owns the next update on ____?

Cover this until after your first attempt

Could you confirm H2's return-to-service status? The service visit ended at eleven, but I do not see the laboratory lead's confirmation that testing can resume. I need to update the clinic accurately. Please distinguish the completed service visit from any checks or release decision still outstanding, and tell me who owns the next status update.

Second round

Ask whether the instrument's illuminated screen proves it is available for testing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

Turnaround times and realistic updates

SHARED CASE

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

Role A | Responding professional

Explain laboratory timing without inventing a guaranteed deadline. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Clinic coordinator: count two hours from the 08:00 collection and ask why no report is available.

Useful expressions

Our quoted interval runs from ___ to ___.

___ is the reference point, not ___.

The current estimate is affected by ___; ___ remains unconfirmed.

Cover this until after your first attempt

The quoted two-hour turnaround is measured from laboratory receipt, not collection. T5 was received at nine after collection at eight. That distinction changes the reference point, but the current backlog means I still cannot promise a report at eleven. The section lead can provide the latest estimate; please treat it as an estimate unless confirmed otherwise.

Second round

Ask whether adding STAT to your telephone message independently changes the recorded order priority.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Preliminary, final, and amended reports

SHARED CASE

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

Role A | Responding professional

Repair an inaccurate version statement without interpreting results. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Clinic nurse: you have the ten o'clock printout and need to identify the current version.

Useful expressions

I incorrectly called ___ final; it was ___.

The version issued at ___ supersedes ___.

Please confirm which report your team is viewing.

Cover this until after your first attempt

I need to correct my earlier description. The ten o'clock report was preliminary, not final. The final report was issued at two and supersedes that earlier version for this request. Please confirm that your team is viewing the two o'clock report. I can clarify the version and status, but clinical interpretation belongs with the responsible clinician.

Second round

Ask whether a value inside a reference interval settles the whole clinical question.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Critical notifications and acknowledged handoffs

SHARED CASE

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

Role A | Responding professional

Communicate a fictional result accurately and confirm its receipt by the stated recipient. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Dr Shah: identity and receiving authority are verified. Initially repeat potassium 6.1 mmol/L for C7; after correction, repeat C7, potassium 6.4 mmol/L and accept the notification.

Useful expressions

For verified case __: [analyte], [value], [units].

You repeated __; the reported value is __. Please read it back.

That read-back matches. Please confirm you accept this notification.

Cover this until after your first attempt

Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter. You repeated six point one; it is six point four. Please read back the complete result. That matches, thank you. The report was issued at two ten; I'll record your receipt at two twelve and contact extension 214.

Second round

Ask whether six point four proves a diagnosis. The technician preserves the reported value and leaves clinical interpretation to the clinician.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Referral laboratories and outstanding work

SHARED CASE

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

Role A | Responding professional

Give a send-out handoff with a clear remaining task and accepted owner. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

Eli: accept the acceptance enquiry and three o'clock update, keeping result timing unknown.

Useful expressions

The manifest lists __; the receiving laboratory has confirmed __.

The outstanding question is __ for accession __.

Please confirm you accept both __ and __.

Cover this until after your first attempt

Eli, X8 is listed on Monday's manifest, and parcel delivery was confirmed Tuesday. We still lack the referral laboratory's accession acceptance for X8, so a result date is not established. Please take the acceptance enquiry and the clinic update agreed for three. Confirm that you have both tasks; parcel delivery alone does not close this send-out follow-up.

Second round

Ask whether a courier signature counts as referral-laboratory accession acceptance.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

Lesson conversations and speaking practice

The following sections match 05 and 06 on the industry webpage and in the learner workbook. Each lesson has three ten-turn conversations and two bounded speaking scenarios. These are original fictional teaching scripts, not transcripts of real people.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A314 or A341

Fictional laboratory call using the accession case.

01 • Clinic caller: I'm asking about A341. Has it reached the laboratory, or is it still with the courier?

02 • Technician: Our log lists accession number A314. Let's clarify it before discussing specimen status.

03 • Clinic caller: Both came from our clinic, so I assumed the two references described the same case.

04 • Technician: The clinic location doesn't establish a match. Could you check the reference on your request?

05 • Clinic caller: The requisition says A314. I transposed the last two digits in my note.

06 • Technician: Thank you. Please repeat the full identifier so we can confirm the enquiry accurately.

07 • Clinic caller: A314. I'm asking whether that specimen's receipt is recorded, not whether a result is complete.

08 • Technician: Following our verification checks, I can confirm receipt for A314. Testing status is a separate question.

09 • Clinic caller: I'll correct my enquiry note to A314 and keep the question limited to receipt.

10 • Technician: That resolves the reference mismatch. Please use A314 for any further enquiry about this case.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

An order without a registered specimen

Fictional electronic order exists, but laboratory receipt has not been confirmed.

01 • Clinic caller: The order appears in our system, so why can't I see an accession receipt?

02 • Technician: An electronic order and a received specimen are different things. I can see the request, not confirmed receipt.

03 • Clinic caller: Does that mean the specimen is lost somewhere between the clinic and the laboratory?

04 • Technician: We haven't established that. The order alone doesn't tell us whether collection or dispatch occurred.

05 • Clinic caller: I'll check the clinic's collection and courier records instead of assuming the laboratory has it.

06 • Technician: Please use the order reference when you check. That helps us compare the same enquiry.

07 • Clinic caller: Can you mark received while we look, just so the order stays visible to staff?

08 • Technician: Received would be inaccurate. The order can remain visible with specimen receipt unconfirmed.

09 • Clinic caller: Understood. I'll report that distinction to the clinician rather than say testing has begun.

10 • Technician: I'll keep the receipt enquiry open under the order reference while your clinic checks its records.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

One request, an additional container

Fictional receiving enquiry about a second container; intended relationship is unresolved.

01 • Technician: A second container arrived with paperwork mentioning the same request. Was it intended as an additional specimen?

02 • Clinic coordinator: I need to check with the collection team. The courier sheet doesn't explain the relationship.

03 • Technician: Please don't infer that it replaces the first specimen simply because it arrived later.

04 • Clinic coordinator: I won't. Do you need the intended relationship or confirmation of a particular test result?

05 • Technician: The relationship is the question. No result status is being discussed in this call.

06 • Clinic coordinator: I'll ask the collection team which request the second container was intended to accompany.

07 • Technician: Our receiving lead will review the specimen label and paperwork before resolving this accessioning discrepancy.

08 • Clinic coordinator: Should I describe both as duplicates in the clinic note while we check?

09 • Technician: Not yet. Duplicate would be a conclusion about their relationship, which remains unresolved.

10 • Clinic coordinator: I'll describe a second container with an unclear relationship and get the collection team's account.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Accessioning and unambiguous identifiers

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: your note says A341; check the actual request when asked.

Second round: Say both cases came from the same clinic and ask whether that is enough to match them.

Scenario 2 | A shared surname on two requests

Two fictional requests have the same surname but different accession references. A caller uses the surname only. The technician can request the full required verification; neither case status has been disclosed.

Role A: Technician: explain why the surname does not identify one case.

Role B: Caller: you have the request reference but initially omit it.

Possible opening: I'm checking the Morgan sample from this morning.

Success checks

- Ask for the required reference and identity verification.
- Avoid choosing a case by arrival order.
- Read back the established reference before discussing its status.

Add a complication: The caller assumes the earlier arrival must be the intended case.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Too little specimen, not a low result

Fictional Q6 call using the lesson case.

01 • Clinic coordinator: The note says insufficient quantity. Does that mean the patient's measured level was low?

02 • Technician: No. It describes the amount of specimen available for Q6, not a measured patient result.

03 • Clinic coordinator: So there isn't a number I should pass to the clinician from this request?

04 • Technician: Correct. Our acceptance criteria aren't met. The lead requested recollection for Q6; no result is reportable.

05 • Clinic coordinator: Does the request tell us when or how to collect another specimen?

06 • Technician: This call provides no collection instructions. Your care team needs to arrange that under its procedures.

07 • Clinic coordinator: I'll pass the documented request to the clinician rather than tell the patient a result was low.

08 • Technician: Please include Q6 so the team knows which request the recollection message concerns.

09 • Clinic coordinator: Can we say the original collector caused the insufficient amount to avoid confusion?

10 • Technician: The cause isn't established. Report the documented quantity issue without assigning blame for it.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Hemolysis is not a diagnosis

Fictional enquiry about a specimen comment; method-specific assessment remains with the laboratory lead.

01 • Clinic nurse: The specimen comment mentions hemolysis. Can you tell me what the patient has?

02 • Technician: The comment describes a specimen condition. It isn't, by itself, a diagnosis of the patient.

03 • Clinic nurse: Does it mean every test from that collection must be considered unusable?

04 • Technician: That depends on the laboratory's assessment for the relevant tests. I can't generalize from this comment.

05 • Clinic nurse: Could you ask the laboratory lead which requested tests the recorded decision covers?

06 • Technician: Yes. I'll ask for the scope of the decision under the accession reference.

07 • Clinic nurse: Please don't send a general recollection message if only one request is affected.

08 • Technician: I'll relay the lead's documented decision, including its scope, rather than invent broader instructions.

09 • Clinic nurse: We need that distinction before we explain the situation to the ordering clinician.

10 • Technician: I'll mark your question as decision scope and ask the lead to clarify it with your team.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Recollection requested, arrival unknown

Fictional specimen-status call; clinic says a new collection was planned, not confirmed.

01 • Technician: We're following up on the recollection request for S12. Has a new specimen been obtained?

02 • Clinic coordinator: It was planned for this morning, but I haven't confirmed whether the appointment took place.

03 • Technician: Then collection and laboratory receipt both remain unconfirmed. I'll avoid marking a new specimen received.

04 • Clinic coordinator: Could you tell the clinician that the plan is in place, even though completion is unknown?

05 • Technician: Yes, provided I label it planned. That wording doesn't imply the collection actually happened.

06 • Clinic coordinator: I'll check the attendance and collection records with our team and return the verified status.

07 • Technician: Please use S12 and distinguish the original specimen from any replacement in your reply.

08 • Clinic coordinator: Should I say the original request is resolved because we've arranged the appointment?

09 • Technician: No. The new appointment doesn't establish a suitable replacement or completed testing.

10 • Clinic coordinator: Understood. I'll report the confirmed stage after checking, rather than close the recollection enquiry early.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Specimen suitability and recollection language

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic coordinator: you think insufficient quantity refers to a low measured result.

Second round: Ask whether the collector definitely caused the problem; the cause is not established.

Scenario 2 | One rejected request, another still pending

A laboratory lead rejected specimen use for request J1 under documented acceptance criteria. Request J2 has no decision yet. The clinic assumes both were rejected. No technical cause or collection directions are supplied.

Role A: Technician: limit the rejection statement to J1.

Role B: Clinic nurse: ask whether J2 also needs recollection.

Possible opening: Should we tell the patient that both tests need another sample?

Success checks

- State J1's documented rejection.
- Keep J2's decision pending.
- Ask the lead to clarify J2 without independently ordering recollection.

Add a complication: Both requests share a collection visit, but their suitability decisions differ in status.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Finished on the instrument, awaiting review

Fictional clinic enquiry using the quality-control case.

01 • Clinic caller: The analyzer has finished, so can you send the final results to our team?

02 • Technician: The analytical run is complete, but quality-control review is unresolved and release hasn't been authorized.

03 • Clinic caller: Our clinician is waiting. Could you at least call the results final on the status screen?

04 • Technician: Final would be inaccurate. I can report completed testing with review and release still outstanding.

05 • Clinic caller: Does that mean all the numbers must be wrong, or that nobody has looked?

06 • Technician: Neither conclusion follows from this status. The laboratory lead is reviewing the issue.

07 • Clinic caller: When will that review finish? We need a definite time for the patient.

08 • Technician: I don't have a confirmed release time. I'll pass the timing concern to the lead.

09 • Clinic caller: Please make the difference explicit in your update, so reception doesn't promise a finished report.

10 • Technician: I will: run complete, quality review unresolved, and no release authorization confirmed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Control material is not a patient result

Fictional new colleague misreads the source of an entry; no numerical results.

01 • Technician: The entry you quoted is labeled control material, not a patient specimen result.

02 • Colleague: I saw it beside the run record and assumed it belonged to the patient accession.

03 • Technician: The control entry monitors the testing process. Its position doesn't make it a patient result.

04 • Colleague: Then I shouldn't copy that entry into a patient-status message or discuss its clinical meaning.

05 • Technician: Correct. First establish the source and record type; keep the run review with the lead.

06 • Colleague: Could you show me the label that distinguishes the two records, without interpreting the control?

07 • Technician: The source field says control material here; the patient record uses its accession reference.

08 • Colleague: I see the distinction now. My question concerns record identity, not whether performance is acceptable.

09 • Technician: I'll ask the lead to review any performance question. We've resolved which entry you were reading.

10 • Colleague: Thanks. I'll withdraw the unsent patient-status wording and check the correct accession record instead.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A run includes more than one request

Fictional result-status query; one reviewed request does not establish the whole run's status.

01 • Clinic nurse: We received a final report for one request. Does that mean everything in the run is released?

02 • Technician: Not necessarily. A final report identifies its own request; it doesn't confirm every other request's status.

03 • Clinic nurse: Could you check the two accession references separately rather than answer about the instrument generally?

04 • Technician: Yes. The first shows released; the second remains under review, with no release time confirmed.

05 • Clinic nurse: I had assumed the same instrument meant the same reporting stage for both.

06 • Technician: The records show otherwise here. Shared testing equipment doesn't make their statuses interchangeable.

07 • Clinic nurse: Please put the second reference first in the update, because that's the one our clinician needs clarified.

08 • Technician: I'll report its review status explicitly and keep the already released request separate.

09 • Clinic nurse: Should I say there's been a new failure, given that one report is delayed?

10 • Technician: No cause is supplied. A different release stage doesn't by itself establish a new failure.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Quality-control status and reportable results

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: you need a status but initially equate a finished run with final results.

Second round: Ask whether the quality review proves the patient results are inaccurate.

Scenario 2 | Review assigned but not completed

The laboratory lead has assigned quality review to reviewer Ana. Ana has acknowledged the task but has not reported a decision. A colleague wants to mark the run released.

Role A: Technician: distinguish review ownership from release permission.

Role B: Colleague: you believe Ana's acknowledgment finishes the review.

Possible opening: Ana accepted it, so can I mark the results released now?

Success checks

- Specify that Ana accepted the review task.
- Keep the review decision pending.
- Avoid interpreting control values or authorizing release.

Add a complication: The colleague quotes the word accepted without naming what was accepted.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

The engineer left at eleven

Fictional service-status call using the H2 case.

01 • Technician: The maintenance record shows the visit ended at eleven. Is H2 confirmed back in service?

02 • Laboratory lead: Not yet. The completed visit doesn't establish the laboratory's readiness decision for testing.

03 • Technician: The clinic asked for a status. Should I say the instrument is still unavailable?

04 • Laboratory lead: Yes, that is the current status. Do not describe the service visit as permission to resume.

05 • Technician: They also asked whether an illuminated screen means the problem has been resolved.

06 • Laboratory lead: A visible screen doesn't establish our required readiness checks. Keep the update to confirmed status.

07 • Technician: Who will provide the next update so the clinic isn't relying on another visual guess?

08 • Laboratory lead: I will. I haven't confirmed a time, so please don't invent one for them.

09 • Technician: I'll say H2 remains unavailable, the service visit is complete, and your review is outstanding.

10 • Laboratory lead: That is accurate. Send equipment-operation questions to me rather than attempting to explain the repair.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Received does not mean verified

Fictional reagent shipment; lot V8 received but lot verification is incomplete.

01 • Technician: The new reagent lot V8 is here. Has its verification status been confirmed?

02 • Stock coordinator: Receipt is logged, but the required lot verification is still pending with the laboratory lead.

03 • Technician: Then I'll describe it as received, not available for testing on the strength of delivery alone.

04 • Stock coordinator: Please do. The purchase order's completed status refers to the delivery, not the technical review.

05 • Technician: Could you identify the lot in the update? There is another batch with similar packaging.

06 • Stock coordinator: Yes, I'll use V8 explicitly and avoid referring to it as just the new box.

07 • Technician: I also need to know whether a different lot is available; can you confirm that?

08 • Stock coordinator: I can't confirm another lot's status from this entry. The lead needs to check that separately.

09 • Technician: I'll ask the lead about availability without treating the two batches as interchangeable.

10 • Stock coordinator: That keeps the stock enquiry precise and avoids turning a receipt record into a use decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

A service reference without patient details

Fictional support call about a display message; no patient data or repair steps are shared.

01 • Technician: I'm following up on service reference H19 for analyzer H2. Has your engineer accepted the enquiry?

02 • Service coordinator: Yes, an engineer is assigned, but we haven't confirmed a visit or resolution time.

03 • Technician: Our clinic team needs an update. Can I quote an assigned engineer rather than a scheduled visit?

04 • Service coordinator: Exactly. Assignment confirms ownership, not travel arrangements or completion of the repair.

05 • Technician: Do you need patient results in this call, or only the equipment reference and reported display message?

06 • Service coordinator: Only the approved equipment information for this enquiry. Don't send patient details with the service reference.

07 • Technician: I'll use the designated support channel and the description already recorded by our lead.

08 • Service coordinator: Please keep the message wording exact rather than paraphrase it as a confirmed technical diagnosis.

09 • Technician: Understood. Engineer assigned, visit unconfirmed; the cause remains with the service review.

10 • Service coordinator: I'll update the laboratory lead through the existing ticket when there is a confirmed scheduling change.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Reagents, instruments, and service coordination

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Laboratory lead: you have not confirmed return to service; ask what the technician needs for the clinic update.

Second round: Ask whether the instrument's illuminated screen proves it is available for testing.

Scenario 2 | A maintenance date from the wrong instrument

A colleague quotes a completed maintenance record for H1 while answering an enquiry about H2. H2 remains unavailable. The laboratory lead can review both references.

Role A: Technician: identify the scope mismatch and request H2's actual status.

Role B: Colleague: you saw a similar instrument name and assumed the record applied.

Possible opening: That maintenance entry names H1; the clinic is asking about H2.

Success checks

- Repeat both instrument references clearly.
- Avoid transferring one instrument's completion status to another.
- Request the lead's H2 status without giving operating advice.

Add a complication: Both instruments are in the same room, which the colleague treats as confirmation.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Which clock starts at nine?

Fictional T5 timing call using the lesson case.

01 • Clinic coordinator: T5 was collected at eight. Doesn't the two-hour turnaround time mean reporting by ten?

02 • Technician: This service measures turnaround from laboratory receipt to reporting. Receipt was at nine, not eight.

03 • Clinic coordinator: Then should I promise the clinician a report at eleven instead of ten?

04 • Technician: No. The backlog affects the estimate, so changing the starting point doesn't guarantee eleven.

05 • Clinic coordinator: Could the section lead provide a current estimate rather than just repeat the usual interval?

06 • Technician: Yes, I'll ask for the latest timing information with T5's reference and receipt time.

07 • Clinic coordinator: We need to distinguish an overdue prediction from a specimen that hasn't arrived.

08 • Technician: Receipt is confirmed. The open question is reporting time, not whether the laboratory has T5.

09 • Clinic coordinator: I'll explain that to the clinician and avoid describing collection time as your receipt time.

10 • Technician: I'll pass on the timing query and keep any new estimate clearly labeled.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

The add-on is a new request

Fictional clinic requests extra testing on an existing specimen; no technical suitability decision.

01 • Clinic nurse: The specimen is already with you. Can we add the extra test without another discussion?

02 • Technician: An add-on request still needs the appropriate order and specimen-suitability review.

03 • Clinic nurse: I thought an existing accession meant every additional test could use the same material.

04 • Technician: It identifies the case, but doesn't establish that the specimen meets every new test's requirements.

05 • Clinic nurse: Could you send the add-on request to the section lead for that review?

06 • Technician: Yes. I'll keep it linked to the original accession while distinguishing the additional requested test.

07 • Clinic nurse: Please tell us whether it was accepted rather than assuming the original testing approval covers it.

08 • Technician: That's the status I'll request. Acceptance and reporting time will need separate confirmation.

09 • Clinic nurse: We haven't told the patient to return for another collection; that decision isn't made.

10 • Technician: I'll preserve that uncertainty and report the lead's decision, without adding collection instructions myself.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

A priority label needs confirmation

Fictional clinician enquiry says urgent; recorded order priority has not been checked.

01 • Technician: Your message says STAT priority, but I need to clarify the recorded order priority.

02 • Clinic coordinator: The clinician asked for an urgent update. I may have used STAT to mean that.

03 • Technician: An urgent request for information isn't automatically a changed testing priority. Let's distinguish them.

04 • Clinic coordinator: We need the current status first, and the clinician can clarify any priority request.

05 • Technician: I'll route that question to the laboratory lead with the order reference.

06 • Clinic coordinator: Please don't change the clinical request based only on my shorthand in this telephone message.

07 • Technician: I won't. I'll describe your message as an urgent status enquiry, with order priority to verify.

08 • Clinic coordinator: Does that wording mean you are deciding it isn't clinically urgent?

09 • Technician: No. I'm not making that assessment; I'm identifying the information that needs the responsible team's clarification.

10 • Clinic coordinator: Thank you. I'll ask the clinician to confirm the intended priority through the proper order route.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Turnaround times and realistic updates

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic coordinator: count two hours from the 08:00 collection and ask why no report is available.

Second round: Ask whether adding STAT to your telephone message independently changes the recorded order priority.

Scenario 2 | A revised estimate after a backlog update

At 10:00 the section lead estimated reporting at noon. At 11:00 the lead says the estimate is no longer supported because the backlog changed. No replacement time is confirmed; a status update is agreed for 12:30.

Role A: Technician: correct the estimate without inventing a new deadline.

Role B: Clinic coordinator: you planned a noon results discussion.

Possible opening: We scheduled around noon. Has the report time changed?

Success checks

- Identify noon as the superseded estimate.
- Keep the new report time unknown.
- State that 12:30 is a status-update commitment.

Add a complication: The coordinator treats 12:30 as the replacement report deadline.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

The two o'clock version

Fictional correction call using the lesson case.

01 • Technician: I need to correct my wording. The ten o'clock preliminary report wasn't final.

02 • Clinic nurse: We printed that version for the clinician. Is there now a different report?

03 • Technician: Yes, the final report was issued at two and supersedes the preliminary version for this request.

04 • Clinic nurse: Please give the issue time again so I can identify the correct document.

05 • Technician: Two o'clock is the final version. I can clarify the version, not interpret its findings.

06 • Clinic nurse: I have it open now. The heading and issue time both match your description.

07 • Technician: Could you alert the clinician that the earlier printout was preliminary and has been superseded?

08 • Clinic nurse: I will. Should I say the newer report proves the patient has no problem?

09 • Technician: No. That is a clinical conclusion the report's final status does not establish.

10 • Clinic nurse: Understood. I'll identify the current version and ask the clinician to interpret it with the patient.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

A reference interval isn't the whole answer

Fictional patient-support call; no values or diagnoses are supplied.

01 • Clinic nurse: A caller asks whether being within the reference interval means every concern is ruled out.

02 • Technician: The interval is a comparison range for that result, not a conclusion about every clinical concern.

03 • Clinic nurse: Could you confirm the report's wording while leaving the patient's interpretation question with the clinician?

04 • Technician: Yes. I can identify the interval printed on the report and the version being discussed.

05 • Clinic nurse: The caller also compares it with a report from another laboratory using a different range.

06 • Technician: Different methods and reference populations may use different intervals; the clinician should review the actual reports.

07 • Clinic nurse: Please don't tell them one laboratory must be wrong simply because the ranges differ.

08 • Technician: I won't. A difference in printed ranges doesn't establish an error by itself.

09 • Clinic nurse: I'll arrange the clinician's explanation and make sure both reports are available for that discussion.

10 • Technician: That gives the clinician the sources and preserves the distinction between comparison information and clinical interpretation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

An amended report needs a notified recipient

Fictional laboratory has issued an amended report; receiving clinic has not acknowledged the notification.

01 • Technician: I'm contacting you about an amended report for request R22 issued this afternoon.

02 • Clinic coordinator: We already have a final report from yesterday. Is this just another copy of that one?

03 • Technician: No. Amended means an issued report has been revised through the laboratory's correction process.

04 • Clinic coordinator: Can you identify the version without discussing patient information on this unverified line?

05 • Technician: Yes. We'll use the established verified contact route for the report details and notification.

06 • Clinic coordinator: I can arrange that route now. Should I mark the clinician notified before we connect?

07 • Technician: Not yet. Arranging a connection doesn't establish that the responsible recipient received the amendment.

08 • Clinic coordinator: I'll connect the designated recipient and keep notification pending until the handoff is acknowledged.

09 • Technician: Thank you. The amendment's clinical meaning belongs with the clinician, but its changed version must be clear.

10 • Clinic coordinator: I'll identify the message as an amendment notification, not a routine duplicate report.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Preliminary, final, and amended reports

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic nurse: you have the ten o'clock printout and need to identify the current version.

Second round: Ask whether a value inside a reference interval settles the whole clinical question.

Scenario 2 | A portal upload mistaken for clinical review

A final report is visible in the portal. The ordering clinician has not confirmed reviewing it or contacting the patient. A caller assumes visibility means the clinical discussion is complete.

Role A: Technician: explain report availability without implying clinical review.

Role B: Clinic coordinator: ask which follow-up task still needs a named owner.

Possible opening: It's in the portal, so can we close the patient-explanation task?

Success checks

- Separate report availability, viewing, and explanation.
- Do not interpret the findings.
- Request confirmation of the clinical follow-up owner.

Add a complication: The caller treats a portal view count as evidence of understanding.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Six point four, not six point one

All facts fictional: Lumen designates C7 potassium 6.4 mmol/L critical under its own criteria. Identity and Dr Shah's receiving authority are already verified. Report issued 14:10, call at 14:12, contact extension 214. No universal threshold, diagnosis, or treatment is implied.

01 • Technician: Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter.

02 • Dr Shah: C7, potassium, six point one millimoles per liter. Have I heard that correctly?

03 • Technician: It's six point four, not six point one. Please repeat the complete result.

04 • Dr Shah: C7, potassium, six point four millimoles per liter. Is that the value on the report?

05 • Technician: That matches, thank you. The report was issued at two ten this afternoon.

06 • Dr Shah: Is two ten the report's issue time, rather than the specimen collection time?

07 • Technician: Yes, two ten is the issue time. This call is taking place at two twelve.

08 • Dr Shah: Please record Dr Shah as the recipient. I've received C7's result, and my extension is 214.

09 • Technician: I'll record your acknowledgment at two twelve and extension two one four. Is that correct?

10 • Dr Shah: Yes, extension two one four. I have the potassium result and will review the report now.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

The escalation contact accepts S9

Fictional Lumen case S9: sodium 121 mmol/L is designated critical by this lab. Report issued 11:06. First contact was unanswered at 11:07; immediate local escalation reached verified receiving nurse Elena at 11:08, extension 305. These are case facts, not a required waiting interval. No universal threshold or treatment is implied.

01 • Technician: Elena, Noor at Lumen laboratory. The first line was unanswered, so I'm calling about S9.

02 • Nurse Elena: Elena speaking. I'm covering S9 and can take the result. Please go ahead.

03 • Technician: The critical sodium result is one hundred twenty-one millimoles per liter, issued at eleven oh six.

04 • Nurse Elena: S9, sodium, one hundred twenty-one millimoles per liter. Please check that against the report.

05 • Technician: That read-back matches. Have you received the result and accepted the notification for S9?

06 • Nurse Elena: Yes, I have. Please use my name, Elena, and extension 305 for this contact.

07 • Technician: Elena, extension three zero five. This call is at eleven oh eight; the earlier attempt was unanswered.

08 • Nurse Elena: That's correct. Eleven oh eight is when I received it, not the earlier call time.

09 • Technician: I'll enter this acknowledgment separately in the notification log. The report is available under S9.

10 • Nurse Elena: Thank you. I have the result and units, and I'll bring the report to the care team.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

The unit needs correcting too

Fictional Lumen case G4: glucose 48 mg/dL, meaning milligrams per deciliter, designated critical under this lab's criteria. Identity and receiving nurse Morgan's authority are verified. Report issued 13:40; Morgan's extension is 306. No universal threshold, diagnosis, or treatment is supplied.

01 • Technician: Morgan, Noor from Lumen laboratory. G4's critical glucose result is forty-eight milligrams per deciliter.

02 • Nurse Morgan: G4, glucose, forty-eight millimoles per liter. Is that the complete result you reported?

03 • Technician: The units are milligrams per deciliter, not millimoles per liter. The value is forty-eight.

04 • Nurse Morgan: I misheard the units. Please say the whole result once more so I can check it.

05 • Technician: For G4: glucose, forty-eight milligrams per deciliter. Please read that back to me.

06 • Nurse Morgan: G4, glucose, forty-eight milligrams per deciliter. That's the result I have recorded now.

07 • Technician: That read-back matches the report issued at one forty. Please confirm who is accepting it.

08 • Nurse Morgan: Morgan. I accept the notification, and you can reach me on extension 306.

09 • Technician: Morgan, extension three zero six. I'll record your acknowledgment and the corrected units; is that contact right?

10 • Nurse Morgan: Yes, three zero six. I have G4's glucose result in milligrams per deciliter.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Critical notifications and acknowledged handoffs

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Dr Shah: identity and receiving authority are verified. Initially repeat potassium 6.1 mmol/L for C7; after correction, repeat C7, potassium 6.4 mmol/L and accept the notification.

Second round: Ask whether six point four proves a diagnosis. The technician preserves the reported value and leaves clinical interpretation to the clinician.

Scenario 2 | A decimal lost in the repeat

Fictional Lumen case M2 has magnesium 0.8 mg/dL, meaning milligrams per deciliter, designated critical by this lab. Case identity and Dr Rao's receiving authority are verified. Dr Rao initially repeats eight, omitting zero point. No diagnosis, treatment, or universal threshold is supplied.

Role A: Technician: state M2, magnesium, zero point eight milligrams per deciliter; correct the repeat and verify the final read-back.

Role B: Dr Rao: initially say eight, then repeat the complete corrected result and accept receipt.

Possible opening: For verified case M2, Lumen's critical magnesium result is zero point eight milligrams per deciliter.

Success checks

- Correct eight to zero point eight without converting units.
- Obtain M2, magnesium, zero point eight milligrams per deciliter in one read-back.
- Confirm the accurate repeat and recipient acknowledgment without interpreting the result.

Add a complication: Dr Rao asks whether eight and zero point eight are merely different ways of saying the same value; they are not.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A delivered parcel with acceptance still open

Fictional X8 shift handoff using the lesson case.

01 • Outgoing technician: Send-out test X8 appears on Monday's shipping manifest. Tuesday's parcel delivery is confirmed, but accession acceptance isn't.

02 • Incoming technician: I'll take the enquiry. Does the clinic have a promised result time or only an update time?

03 • Outgoing technician: Only an update at three. There is no confirmed result date from the referral laboratory.

04 • Incoming technician: I accept that clinic update too. I'll ask the referral laboratory specifically about X8's acceptance.

05 • Outgoing technician: Please don't treat the courier signature as proof they registered every item in the parcel.

06 • Incoming technician: I won't. Parcel delivery and item-level acceptance remain separate in the handoff.

07 • Outgoing technician: The clinic also asked whether this means X8 is lost. We haven't established that.

08 • Incoming technician: I'll describe an acceptance enquiry, not a lost specimen, unless the receiving team confirms more.

09 • Outgoing technician: Then you own the external check and three o'clock update, with no promised report date.

10 • Incoming technician: Accepted. I'll record the response and keep unresolved items visible for the next shift.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A manifest entry without receipt confirmation

Fictional send-out coordinator compares a shipment list with the external receiving log.

01 • Technician: Our manifest lists S4 and S5, but your acknowledgment names only S4.

02 • Referral coordinator: I can confirm S4's registration. I need the receiving team to check S5 separately.

03 • Technician: Does the acknowledgment intentionally exclude S5, or is that reason still unknown?

04 • Referral coordinator: The reason is unknown. I shouldn't call S5 rejected simply because it isn't mentioned.

05 • Technician: Could you open an item-level enquiry using the manifest reference and the two separate identifiers?

06 • Referral coordinator: Yes. I'll ask the receiving team to compare the physical receipt information and registration records.

07 • Technician: Please keep S4's confirmed status unchanged while you investigate the separate S5 question.

08 • Referral coordinator: Agreed. I won't turn an unresolved item into an assumption about the entire shipment.

09 • Technician: Our clinic update will say S4 confirmed and S5 under receipt enquiry, without predicting a result date.

10 • Referral coordinator: That matches what I can establish now. I'll return the receiving team's finding when available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

Custody wording has a specific purpose

Fictional laboratory staff clarify documentation terminology without teaching collection or forensic procedures.

01 • Technician: The clinic asked for chain-of-custody documentation. Is that just another name for our routine courier receipt?

02 • Quality coordinator: No. Chain of custody is a specific documented record of possession and transfers where required.

03 • Technician: So I shouldn't rename a delivery signature as a complete chain-of-custody record.

04 • Quality coordinator: Correct. We need to establish the purpose and applicable requirements through the responsible laboratory team.

05 • Technician: Can you review what the clinic is requesting before we describe the documentation as sufficient?

06 • Quality coordinator: Yes. Send the enquiry through the designated record route, with the request purpose stated.

07 • Technician: They haven't told us the purpose yet. I'll obtain that clarification instead of assuming it.

08 • Quality coordinator: Good. Don't promise that a routine send-out meets requirements that haven't been identified.

09 • Technician: I'll explain that the courier receipt confirms its stated delivery event, not every possible documentation requirement.

10 • Quality coordinator: That is the useful distinction. I'll take the documentation review once the intended purpose is clarified.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Referral laboratories and outstanding work

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Eli: accept the acceptance enquiry and three o'clock update, keeping result timing unknown.

Second round: Ask whether a courier signature counts as referral-laboratory accession acceptance.

Scenario 2 | The receiving laboratory changes its contact person

Referral laboratory contact Ana has moved roles. Send-out Z3 remains pending. Eli has not accepted the query. The receiving laboratory's service coordinator can assign a new owner; a clinic update is due at 16:00.

Role A: Technician: request a named new contact without claiming the test stopped.

Role B: Service coordinator: you can accept the ownership enquiry, but cannot confirm result timing.

Possible opening: Ana no longer handles Z3. Who has accepted the follow-up now?

Success checks

- Separate contact ownership from specimen suitability.
- Keep result timing unknown.
- Preserve the four o'clock update while resolving who owns the external query.

Add a complication: The clinic assumes a staff change means its specimen must be recollected.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

U.S. Bureau of Labor Statistics: Clinical Laboratory Technologists and Technicians

<https://www.bls.gov/ooh/healthcare/clinical-laboratory-technologists-and-technicians.htm>

Centers for Disease Control and Prevention: About Laboratory Quality

<https://www.cdc.gov/lab-quality/php/about/>

Agency for Healthcare Research and Quality: TeamSTEPPS Check-Back (or Repeat-Back)

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/checkback.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Laboratory Technician English

Professional audience: Medical laboratory technicians and clinical laboratory support staff

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Dialogue 01: A barcode scan and a spoken reference disagree

Original fictional setting: Fictional laboratory desk enquiry; a scan and telephone note point to different accessions.

Professional roles: Technician / Receiving lead

Published dialogue (reference script, not my own performance):

Technician: The scan retrieves B207, but the telephone note says B270. I haven't linked the enquiry to either.

Receiving lead: Which source produced each reference? We need to separate the scan from the caller's spoken information.

Technician: The scanned document retrieves B207. A colleague wrote B270 during the call, without a read-back.

Receiving lead: That gives us a discrepancy to check, not proof the caller or the scanner was wrong.

Technician: Could you review the document association while I ask the colleague to clarify the original enquiry?

Receiving lead: Yes. I will check the registered source, and you can establish what the caller actually requested.

Technician: Should we answer the receipt question for B207 because it is the only visible scan?

Receiving lead: Not until the enquiry is identified. A visible reference isn't automatically the reference the caller intended.

Technician: I'll explain that we are clarifying two different identifiers and haven't established their relationship.

Receiving lead: Please keep both sources visible in the review, rather than overwriting one reference with the other.

Technician: Then I'll request a complete read-back from the caller through the verified contact route.

Receiving lead: I'll take the record-association check. We can answer the status question once the intended case is established.

Vocabulary in this exchange: read-back: Repeating communicated details so the sender can check that they were received accurately.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
 2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
 3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
 4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
 5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.
- Use voice replies when available. If I prefer selections, offer four plausible spoken replies and explain my choice before continuing. Do not require written compositions.

END PROMPT

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Receiving lead: I'll take the record-association check. We can answer the status question once the intended case is established.

Vocabulary in this exchange: read-back: Repeating communicated details so the sender can check that they were received accurately.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT